

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°131/MVE-BDBV/09/22/2026

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HIGHLIGHTS (last 24 hours)

Forty-seven (47) new confirmed cases of Ebola Virus Disease (EVD), including 13 community deaths, were reported during the day on September 22, 2026. These cases are distributed in the provinces of Ituri (35 cases),

from North Kivu (11 cases) and South Ubangi (1 case). In addition, 7 deaths within the Ebola Treatment Center (ETC) were recorded.

For the day of September 22, 2026, 328 new samples were analyzed, 47 of which were found to be positive, representing a positivity rate of 14.0%.

No new health zones have been affected in the last 24 hours.

ACCUMULATIVE CASE 7,820	ACCUMULATIVE DEATH CONFIRMED 3,779	LETHALITY 48.3%	PATIENTS IN ISOLATION/CTE 885	ACCUMULATIVE HEALED 1951	TRACKING RATES CONTACTS (Of the Day) 78.3%
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Affected Provinces and Affected Health Zones



7 Provinces affected

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo, Bas-Uélé and
South Ubangi



63 out of 167
health zones affected
(37.7%)

Summary of key points from the last 24 hours

A total of 47 new confirmed cases of Ebola Virus Disease (EVD) have been notified at the end of the day on September 22, 2026, including 13 community deaths. The Ituri province (35 cases) alone accounts for three-quarters of the new confirmed cases, followed by North Kivu with 11 cases and South Ubangi (1 case), which reported its second confirmed case.

In addition, 7 deaths were recorded in the CTEs (4 in Ituri, 2 in North Kivu and 1 in Haut-Uélé), bringing the total number of confirmed deaths for the day to 20.

However, 16 patients (6 in Ituri, 5 in North Kivu, 4 in Haut-Uélé and 1 in Tshopo) were declared cured after obtaining two negative results for Ebola virus disease.

From the start of the epidemic until September 22, 2026, 7,820 confirmed cases, including 3,779 deaths, were recorded in the Democratic Republic of Congo (DRC), representing a case fatality rate of 48.3 %. The epicenter remains the Ituri province, which accounts for 76.7% of reported confirmed cases and also represents 74.5 % of new confirmed cases.

No new health zones have been affected in the last 24 hours.

hours. Thus, 63 out of 167 health zones (37.7%) have been affected in 7 provinces since the start of the epidemic. Ituri has 28 out of 36 affected health zones, followed by North Kivu (16/34), Haut-Uélé (7/13), Tshopo (7/23), Bas-Uélé (3/11), South Kivu (1/34) and South Ubangi (1/16).

Table 1. Distribution of confirmed cases and deaths by affected province, as of September 22, 2026.

Province	New cases confirmed (24h)	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones
Ituri	35	6001	2,751	45.8%	28/36 (77.8%)
North Kivu	11	1449	877	60.5%	16/34 (47.1%)
Haut-Uélé	0	316	130	41.1%	7/13 (53.8%)
Tshopo	0	42	15	35.7%	7/23 (30.4%)
South Kivu	0	3	1	33.3%	1/34 (2.9%)
Bas-Uélé	0	7	4	57.1%	3/11 (27.3%)
South Ubangi	1	2	1	50.0%	1/16 (6.3%)
Total	47	7,820	3,779	48.3%	63/167 (37.7%)

Table 2. Distribution of confirmed cases and deaths by province and zone as of September 22, 2026

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	6001	2751	45.8%	35	10	4	14
Adja	11	1	9.1%				0
Aru	8	6	75.0%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	197	41	20.8%	1	0		0
Boga	2	2	100.0%				0
Bunia	1678	528	31.5%	12	5		5
Damascus	40	9	22.5%				0
Drodro	15	7	46.7%				0
Fataki	81	30	37.0%				0
Gethy	7	3	42.9%				0
Kambala	2	0	0.0%				0
Kilo	34	12	35.3%				0
Komanda	153	93	60.8%	6	2		2
Lita	276	138	50.0%				0
Logo	13	5	38.5%				0
Lolwa	32	8	25.0%	1	0		0
Mahagi	4	1	25.0%				0
Mambasa	23	9	39.1%	1	0		0
Mandima	66	36	54.5%	7	3		3
Mangala	341	149	43.7%				0
Mongbwalu	692	323	46.7%	1	0	1	1
Nia-Nia	269	149	55.4%			1	1
Nizi	762	276	36.2%	1	0		0
Nyankunde	127	37	29.1%			1	1
Rimba	12	4	33.3%				0

Rwampara	1061	359	33.8%	5	0	1	1
Tchomia	75	25	33.3%				0
To ventilate	N / A	493	N / A				0
North Kivu	1449	877	60.5%	11	3	2	5
Blessed	292	204	69.9%	3	0		0
Good	18	7	38.9%				0
Butembo	253	162	64.0%				0
Goma	1	0	0.0%				0
Kalunguta	49	16	32.7%				0
Katwa	548	348	63.5%	4	3	1	4
Kayna	1	0	0.0%				0
Kyondo	32	20	62.5%				0
Lubero	20	10	50.0%				0
Mabalako	24	11	45.8%				0
Manguredjipa	13	4	30.8%	1	0		0
Masereka	25	9	36.0%				0
Musienene	121	65	53.7%				0
Mutwanga	10	2	20.0%	1	0		0
Oicha	15	11	73.3%	1	0		0
Vuhovi	27	8	29.6%	1	0	1	1
Haut-Uélé	316	130	41.1%	0	0	1	1
Boma Mangbetu	36	15	41.7%				0
Dungu	1	1	100.0%				0
Gombari	3	1	33.3%				0
Isiro	99	43	43.4%				0
Pawa	70	33	47.1%	0	0	1	1
Rungu	2	1	50.0%				0
Wamba	105	36	34.3%				0
Tshopo	42	15	35.7%	0	0	0	0
Bafwasende	7	1	14.3%				0
Kabondo	5	1	20.0%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	22	9	40.9%				0
Mangobo	5	3	60.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	7	4	57.1%	0	0	0	0
Buta	1	1	100.0%				0
Viadana	3	1	33.3%				0
Ganga	3	2	66.7%				0
South Ubangi	2	1	50.0%	1	0	0	0
Bulu	2	1	50.0%	1	0	0	0
Total	7820	3779	48.3%	47	13	7	20

*These are the confirmed deaths that occurred in the various CTEs in the province which are being broken down to be classified by health zone.

NA: Not Applicable.

Table 3. Status of alerts notified by province for September 22, 2026

DPS	Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CTE/CT (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	19	5	24	18	3	1	0	21	18
Bas-Uélé	48	0	48	8	0	40	0	8	0
Ituri	1,250	84	1334	108	82	1119	2	175	63
North Kivu	1,238	68	1306	151	68	993	0	219	114
South Ubangi	12	1	13	3	0	10	0	3	0
Tshopo	80	1	81	7	1	73	0	1	0
Total General	2,647	159	2,806	295	154	2,236	2	427	195

Figure 1. Evolution of cases by date of symptom onset over the last 21 days

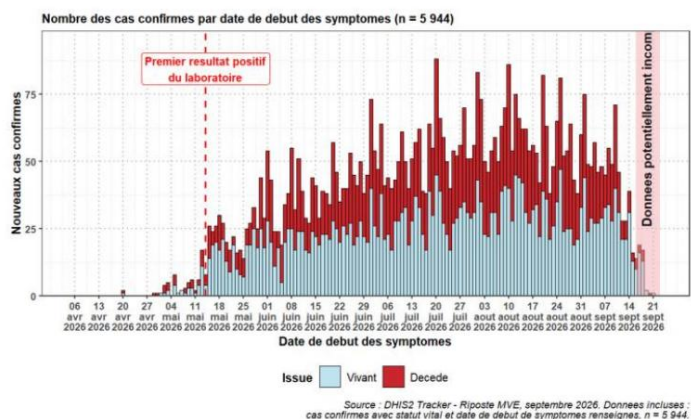
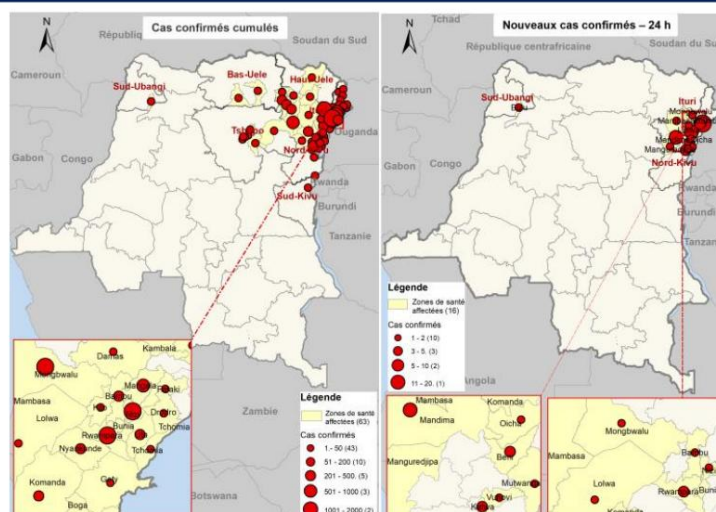


Figure 2. Spatial distribution of confirmed cases (24h and cumulative) by health zone.



1. COORDINATION

1.1. Main Actions

- Holding coordination meetings and providing support to the affected provinces.

2. RESPONSE ACTIONS BY PILLAR

2.1. Epidemiological surveillance

2.1.1. Main Actions

For September 22, 2026, **2,687 (95.8%)** of the **2,806 alerts** registered in 6 provinces that reported them were verified. Following these verifications, **449 alerts were validated as suspected cases**, **427 (95.1%)** were investigated and **195** of the suspected cases investigated were transferred to CT/CTE (Table 3).

Of the **32,745** contacts currently being monitored, **25,652 were seen** in the last 24 hours, representing a follow-up rate of **78.3 %**, lower than the previous day's rate of 84.5%. This rate was 92.7% (458/494) in Tshopo, 86.7% (52/60) in Sud-Ubangi, 81.0% (111/137) in Bas-Uélé, 80.1% (1,560/1,947) in Haut-Uélé, and 78.6% (12 786/16,268) in Ituri and 77.2% (10,685/13,839) in North Kivu. No province had tracked all contacts by the end of the day.

2.2. Update on checkpoint and point of entry (PoE/PoC) activities

2.2.1. Main actions

Five (5) alerts were notified to the PoE/PoC in the last 24 hours, including two (2) in Ituri, at the PoCs of Tchaï (Rwampara Health Zone) and Mudzibala (Bunia Health Zone), and three (3) in North Kivu: two live alerts at the PoCs of Foner Kasindi (Mutwanga Health Zone) and Maboya (Kalunguta Health Zone), and one body intercepted at the PoC of Kasitu (Beni Health Zone). All four (4) live alerts were validated and investigated within 4 hours (100%; 4/4), and three were transferred to the CT/CTE (75.0%; 3/4). The suspect in Tchaï refused transfer, while the body was referred to the morgue for swab (100%; 1/1).

Table 4. Status of key PoE/PoC indicators by province, as of September 22, 2026

Indicators	Ituri	North Kivu	Tshopo	Haut-Uélé	Overall
Report completeness	96.7% (58/60)	100% (20/20)	44.4% (8/18)	100% (8/8)	88.7% (94/106)
% of functional strategic PoE/PoC	100%	100%	100%	100%	100%
Number of people who have switched to PoE/PoC	140 141	95,611	21,487	15,982	273,221
% of travelers screened	97.0%	97.8%	99.3%	99.0%	97.7%
% of travelers who washed their hands	97.0%	97.8%	99.5%	99.0%	97.8%
% of travelers made aware	99.0%	97.8%	100.0%	100.0%	99.0%
Number of alerts notified	2	3	0	0	5
Number of validated live alerts (suspected cases)	2	2	0	0	4
Number of bodies intercepted today % of suspected	0	1	0	0	1
cases transferred to CTE/CT	50.0% (1/2)	100% (2/2)	N / A	N / A	75.0% (3/4)
% of lifeless bodies swabbed/secured	N / A	100% (1/1)	N / A	N / A	N / A
% of suspected cases investigated within 4 hours	100% (2/2)	100% (2/2)	N / A	N / A	100% (4/4)

2.2.2. Challenges

Low completeness of alert entry in DHIS2 Tracker and insufficient mobility in the Rwampara and Bunia health zones (Ituri); in North Kivu, persistent threats against PoC providers in the Butembo axis; in Tshopo, low completeness of reports (44.4%; 8/18); in Bas-Uélé, only 39% of sites are activated (16/41), none of the 11 PoEs are reinforced and mobility, connectivity and water supply problems persist.

2.3. Laboratory

2.3.1. Main actions

- **Ituri: 35 new positive results** (25 living and 10 deaths) out of 190 new samples received and analyzed (**positivity: 18.4%**).
- **North Kivu: 11 new positive results (8 living and 3 dead) out of 130 samples received and analyzed (positivity: 8.5%).**
- **Haut-Uélé: 2 samples received and analyzed**, all were negative.
- **Tshopo: 4 samples received and analyzed**, all were found to be negative.
- **Bas-Uélé: 2 samples received and analyzed** in the laboratory, all were found to be negative.

2.4. Infection Prevention and Control (IPC/EDS)

2.4.1. Main actions

- **In Ituri**, 16 out of 44 rings were opened (36.4%), and 61 out of 64 identified households were decontaminated (95.3%); all 7 health facilities were decontaminated (7/7), and 30 households received hygiene kits in Mongbwalu. Regarding the blood and sanitation services (BSS), 104 alerts were recorded (including 8 postponements from the previous day), 76 BSS were carried out (73.1%), and 71 bodies were swabbed; 19 could not be swabbed (18.3%).
- **In North Kivu**, 26 out of 36 infection control rings (72.2%) were opened, 12 out of 41 households were decontaminated (29.3%), and 22 out of 27 health facilities (81.5%) were decontaminated. Seventeen infection prevention and control (IPC) scorecard assessments were conducted in 17 health facilities with an average score of 18.4%, and the risk assessment of staff exposure classified 6 out of 16 staff members as high risk in Beni. Training of the blood and sanitation teams (BST) was held in Lubero with WHO support, and training of service providers and hygiene workers is continuing in Mabalako, Beni, Butembo, Kalunguta, and Mutwanga with IMA/MOMENTUM support. Regarding BSTs, 54 alerts were received (plus 23 reports from the previous day), 54 bodies were successfully swabbed, and 55 BSTs were carried out.
- **In Tshopo**, decontamination of the DSHKis (20 rooms, 14 latrines, and 3 corridors), the Maison Colorée hotel, and the Kawaki clinic; IPC scorecard assessment of the DSHKis (74%) and the Kawaki clinic (10%); 13 ESS (Emergency Health Services) monitored and supported in 6 health zones, with 152 service providers briefed on standard precautions. Regarding EDS (Emergency Health Services), no alerts were received, nor were any EDS carried out.

2.4.2. Challenges

In Ituri: low opening of the rings (36.4%; 16/44); 19 bodies not swabbed, in Fataki and Mandima due to lack of inputs and teams and elsewhere due to refusal of families; resistance in Bambu, Komanda, Nia-Nia and Mangala and insufficient means of transport in Kilo, Nia-Nia, Mandima and Fataki.

2.5. Holistic care

2.5.1. Main actions

- **In Ituri**, 77 new admissions were recorded during the day, compared to 73 discharges, including 6 recoveries. Thus, 429 patients are hospitalized out of 1015 beds (occupancy rate of 42.3%). 397 out of 429 patients (92.5%) received hot meals and children under LNPE/SLM were fully covered at the CTE (14/14), at the nursery (3/3) and in the community (27/27); 6 suspects left against medical advice from the Mambasa CTE following the HGR fire.
- **In North Kivu**, 95 new admissions were recorded compared to 56 discharges, including 5 recoveries; 377 Patients are hospitalized, including 280 in standardized facilities with 308 beds, representing an occupancy rate of 90.9%; 97 patients (25.7%) remain under the care of social and medico-social establishments outside of the CTE (Centre de Traitement et d'Enseignement). All 87 patients, 12 accompanying persons, and 76 frontline service providers received hot meals (100%).
- **In Haut-Uélé**, 12 new admissions were recorded against 6 discharges (4 recovered). Thus, 56 patients are hospitalized for 138 beds, representing an occupancy rate of 40.6%.
- **In Tshopo**, six new admissions were recorded and one patient was discharged (recovered). Sixteen (16) patients are in isolation out of 31 beds, representing an occupancy rate of 51.6%; 9 confirmed and 20 suspects received a hot meal, and 42/42 (100%) frontline providers also received hot meals.
- **In Bas-Uélé**, no new admissions or discharges were recorded; 7 patients remain in isolation in non-standard CTs, including 1 confirmed at HGR Ganga.

2.5.2. Challenges

Treatment remains partially substandard and capacity is limited: **In Ituri**, Ebola Treatment Centers (ETCs) are saturated with suspected cases in Nizi, Mambasa, Lolwa, Kilo, Mangala, and Mandima. **In North Kivu**, 25.7% of hospitalized patients are treated outside of standardized facilities; no beds are available in Butembo and Katwa. **In Tshopo**, the lack of safe blood and limited technical resources persist. **In Bas-Uélé**, the absence of ETCs and ambulances hinders the isolation of cases.

2.6. Vaccination

2.6.1. Main actions

- **In Tshopo**, 3,874 TPL and PPL were vaccinated out of 3,549 pre-registered (109%), i.e. 33% of the target of 11,703, including 1,909 in Makiso-Kisangani, 513 in Kabondo and 458 in Bafwasende; 4,000 doses were distributed to the ZS and 626 thawed doses remain in stock, of which 126 expire on September 24, 2026 and 500 on October 5, 2026.

2.6.2. Challenges

Low uptake of vaccination among people living with HIV and HIV-positive individuals in Tshopo (33% of the target population), where coverage remains low in Tshopo (36%) and Lubunga (59% of pre-registered individuals), and risk of loss of 126 thawed doses expiring on September 24, 2026; in Bas-Uélé, lack of a micro-plan for vaccination of people living with HIV in the 6 priority health zones and need for additional doses of ERVEBO.

2.7. Continuity of Care

- **In Tshopo**, 29 confirmed cases are under treatment (3 on MBP134 and 26 on remdesivir: 10 recovered, 7 deceased, 1 escaped, and 8 still receiving treatment), and 185 people have received post-exposure prophylaxis (101 completed, 12 interrupted, and 72 ongoing); the 11th and 12th frontline providers have tested positive, the 6th has been relieved of duties, and the temporary closure of the ESS DHKis is recommended due to the increasing number of cases. In Ituri, the development of the recovery plan for the Bunia General Referral Hospital's CSSE continues, where 65 of the 100 staff members (65%) are involved in the response, and a survey on the impact of Ebola on the use of reproductive health services has been launched with support from MDA and Africa CDC

2.8. Risk Communication and Community Engagement (RCCE)

2.8.1. Main actions

- **In Ituri**, 30,338 home visits reached 98,168 people and 1,594 educational talks benefited 29,229 people; 413 mass awareness campaigns reached 11,830 people, 64 community debates (570 participants) and 74 advocacy sessions reached 244 people; 423 community alerts were reported, including 53 deaths, 381 feedback reports were received, of which 353 were resolved (92.7%), and 17 community incidents were reported, of which 5 were resolved (29.4%), for a completeness rate of 32.1% (9/28 health zones); 84 public service announcements were broadcast on 18 radio stations and 14 programs were hosted, and 54 teachers and nearly 1,000 parents of students at the Bakhita School were sensitized to school-based surveillance
- **In North Kivu**, 9,239 home visits reached 30,302 people, and 53 educational talks were held. 3,664 people were brought together, 23 mass communication sessions with 10,939 people and 9 briefings with 251 people; 227 alerts of suspected cases and deaths were notified by community actors out of 450 expected (50.4%), 10 out of 15 feedbacks were resolved within 48 hours (66.7%) and 1,853 out of 4,750 people were engaged in community actions (39.0%); the release of 4 recovered patients from the Ebola Treatment Centers in Musienene, Oicha and Katwa was publicized to strengthen community confidence.
- **In Bas-Uélé**, 217 home visits reached 898 people, 3 educational talks (25 people reached) and 3 mass awareness campaigns reached 228 people in the health zones of Ganga, Poko and Buta; advocacy was carried out with the chief of the Bakete chiefdom and 43 people in his entourage were briefed on Ebola prevention; 12 community alerts were notified and all dealt with and the communication plan for the 10 sub-pillars was validated with UNICEF.

- **In Tshopo**, 218 home visits reached 1,221 people and 2 educational talks (reaching 120 people); 1,064 students (563 girls) and 29 teachers from three schools were sensitized, and a village assembly brought together 498 people.

Baego (267 men and 231 women) and 30 school principals were made aware of the issue; 21 community alerts were notified and all addressed, 2 feedback requests and 2 refusals resolved, 564 posters and 186 leaflets distributed, and 5 radio programs hosted.

2.8.2. Challenges

Persistent community resistance: **In Tshopo**, confirmed cases who escaped from the Ebola Treatment Center (ETC) (20th and 26th cases, Mangobo Health Zone) remain at home, and 10 contacts of the 20th case refuse follow-up. **In Ituri**, data reporting is low (completeness: 32.1%; 9/28), there is a lack of numerical data on interventions around cases, and demotivation of stakeholders due to unpaid fees. In **North Kivu**, resistance persists in the Lubero Health Zone (Vunyakondomi Health Area), a suspected case in Butembo refuses to be transferred to the Treatment Center, CREC stakeholders are demotivated due to unpaid fees for over four months, and community alerts are low (50.4%; 227/450). **In Bas-Uélé**, there is low reporting from Health Zones, a lack of communication credit, and impassable roads.

2.9. Mental Health and Psychosocial Support (MHPSS)

2.9.1. Main actions

- **In Ituri**, 14 of the 19 confirmed cases (73.7%) from the previous day and 114 of the 188 confirmed cases under follow-up (60.6%) benefited from SMSPS interventions, as well as 24 of the 77 new suspects (31.2%) and 108 of the 141 suspects under follow-up (76.6%); 270 caregivers and 61 PPLs were supported, 19 of the 26 recovered (73.1%) were followed up in the community and 46 results were announced, including 14 positive results; 11 of the 28 health zones are covered, 11 children are being followed up in daycare with one new admission and 12 of the 14 households of separated and orphaned children were supported.
- **In North Kivu**, the last 13 confirmed cases (100%) and 201 confirmed cases under follow-up benefited from SMSPS interventions, as well as 105 new suspected cases and 346 suspected cases under follow-up; 66 households were supported for public health actions and 70 PPLs were supported, 8 out of 8 community-followed recovered and 96 results announced, including 13 positive results; the 5 health zones that notified are covered and 176 men, 218 women and 79 children benefited from psychoeducation sessions with the support of UNICEF.
- **In Tshopo**, the last 2 confirmed cases (100%) and the 5 confirmed cases under follow-up, as well as the 5 new suspects and the 8 suspects under follow-up, benefited from SMSPS interventions; 26 people benefited from psychoeducation at the CTE Météo, including 25 accompanying persons and 1 PPL, 3 announcements of results were made, 2 of which were positive, a household was prepared for public health actions in Kabondo, a recovered person was reintegrated and the 3 notified resistances were all broken.

2.9.2. Challenges

- **In Ituri**, low coverage of SMSPS interventions (11 of the 28 health zones covered) and a need to strengthen collaboration between pillars for joint interventions around the latest confirmed cases; **in North Kivu**, a lack of nurseries in 10 affected health zones and insufficient human resources and logistical means for the pillar; **in Tshopo**,

need to train the APS (Auxiliary Security Personnel) in the health zones of Wanie-Rukula and Bafwasende and to support the pillar with communication units and means of transport

2.10. Logistics

2.10.1. Main actions

- Support for the mobility of response teams in all affected provinces; to the Tshopo, receipt of supplies provided by Village Reach, 14 infrared thermometers for vaccination teams (WHO) and laboratory supplies, and continuation of the construction of the CTE of Bafwasende; in Bas-Uélé, construction of a transit center at the Buta General Referral Hospital and continuation of the rehabilitation of the DPS premises with the support of the WHO; corrective maintenance of

vehicles and support for the deployment of IPC inputs, care, communication, PoE/PoC and vaccination.

2.10.2. Security

2.10.3. Main actions

- Continued security monitoring in the affected provinces; in North Kivu, EDS and PCI teams were accompanied in the sub-coordinations of Beni and Katwa (100%) for the burial of 6 confirmed cases and the swab of a lifeless body, and public order was restored without incident at the morgue of the HGR of Beni after the disappearance of a body claimed by a family.

2.11. PSEA (Prevention of Sexual Exploitation and Abuse)

2.11.1. Main actions

- **In Ituri**, 220 people (56 men, 43 women, 62 boys and 59 girls) were sensitized on the PSEAH in the health zones of Bunia, Rwampara and Mongbwalu (traders from the Saïo market, students, taxi drivers and youth groups); visibility materials were received from the WHO and the display of messages and the distribution of leaflets continue in Bunia, Aru, Mongbwalu, Mambasa and Fata.
- **In Haut-Uélé**, CBCMs were installed in Isiro (124 participants), a community survey was conducted in Wamba (35 participants) and 344 beneficiaries were sensitized; **in North Kivu**, 44 young leaders were briefed and 25 people were sensitized to reporting (495555 and 151) in the Beni health zone.
- **At the national coordination level**, 44 staff members (23 men and 21 women) were briefed and all signed the code of conduct, 742 people were sensitized in the community and six partners reported their activities; 41 of the 119 ZS monitored (34.5%) know the reporting mechanism.

2.11.2. Challenges

- Still insufficient coverage of PSEA activities, with only 41 of the 119 health zones monitored (34.5%) aware of the reporting mechanism; lack of mapping of stakeholders and refusal of alignment by some partners; insufficient mass awareness materials and lack of means of transport and computer kits for the pillar.

KEY MESSAGES

- Active and persistent community transmission in the provinces of Ituri, North Kivu, Haut-Uélé, Tshopo and Bas-Uélé. In addition, the province of Sud-Ubangi has recorded a new confirmed case in the last 24 hours, so it is necessary to strengthen response activities in order to break the chain of transmission as quickly as possible.
- Progress that needs to be consolidated has been made in the field of epidemiological investigations: almost all (95.1 %) of suspects were investigated within 24 hours of suspected cases being investigated, although the contact tracing rate fluctuates (78.3%).
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to rapidly interrupt transmission chains: Rapidly increase reception capacities in ETC/CT in North Kivu and strengthen cross-border surveillance with South Sudan.
- Strengthen preparedness and response to Ebola Virus Disease-B in the provinces of the River Corridor and surrounding areas.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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